

Your Arkin record: English summary and translation

Prepared for Tom Klootwijk from the 23-page Arkin record supplied in this conversation.

Source: ECD_822577_I70227739_06-01-2022_tm_heden.pdf. Client number 822577; record number I70227739. Copy issued 10 August 2026.

Scope: Although the cover says the registration began on 6 January 2022, the entries included in this PDF cover 23 July to 7 August 2026. This is a short extract of progress notes, not your entire medical history.

This document contains a plain-English summary followed by a translation of every substantive entry. Page references identify the physical pages of the original 23-page PDF. Dates in the translation use day-month-year. Repeated administrative labels, empty fields and print footers are omitted; repeated clinical observations are retained. The four standard nursing observation questions are explained once. Square brackets identify translation clarifications or unclear source wording. This is an explanatory translation, not a certified translation or an independent clinical assessment.

What the record says overall

The central issue is a disagreement about how to interpret changes in your communication, emotions and activities. Some staff suspected that you might be developing early psychotic symptoms. You and your mother described the changes as recovery: your usual personality, faster thinking and emotions returning after a long period of feeling emotionally flattened.

The record does not present that disagreement as conclusively settled. The psychologist initially considered your explanation plausible, later became more concerned, and still explicitly acknowledged alternative explanations on 6 August. The psychiatrist's brief 3 August entry is more definite in its wording: "First signs of psychosis."

What concerned staff

Staff described occasional difficulty following your abstract or philosophical explanations, shifts between topics, distraction or staring away during conversations, and more irritability when they raised concerns. Some wondered whether statements about inventions, AI and products that could benefit humanity or earn substantial money reflected grandiose ideas. These are recorded interpretations, not an independent assessment of your technical work. [Source pages 12, 15, 17, 20-23]

The 27 July team discussion explicitly says there were no very concrete indications of psychosis, such as delusions or hallucinations. It says the concern arose from the overall change noticed by several staff members, rather than individual behaviours being especially unusual or concerning on their own. This statement belongs to that dated discussion; it does not establish what could or could not have occurred at other times. [Source page 23]

On 7 August a nurse recorded your wish to end financial administration, find a job and possibly rent a home if supported housing was delayed. He wrote that this could be understood in terms of recovery, while also considering behaviour resembling mania as another explanation. He did not establish a diagnosis of mania in that note. [Source page 6]

Your explanation, and your mother's

You said you were reflecting more on your future and values, and that both pleasant and difficult emotions were returning. You said you were showing distraction more openly instead of pretending to follow conversations. Your mother recognized the faster, more abstract thinking and sometimes sharp communication as characteristics you had before becoming unwell. On 27 July she said she was not worried, and trusted you to speak up if things were going badly. [Source pages 21-22]

You also explained that concern about being misinterpreted could make you hold back or respond abruptly. The psychologist acknowledged that staff had not known the earlier version of you. On 6 August you said your main difficulty was how staff were viewing you, rather than the medication increase itself; repeated evaluations made you feel scrutinized and as though you were in a "trial period." [Source pages 19, 22]

Positive observations recorded alongside the concerns

Many entries describe you as friendly, considerate, appropriately engaged, coherent and in a good or normal mood. Several explicitly record no observed change in your mental state or no observed psychotic features during that contact. Your personal care was repeatedly described as in order. Notes mention taking medication, walking, listening to podcasts, programming or developing a game, attending daytime activities, seeing a friend and spending time with your mother. [Source pages 5-18]

The observations vary, sometimes within the same day. For example, the morning note on 23 July describes relaxed and appropriate conversation, while the evening note raises concerns about your communication. The 1 August daytime note records no unusual or incoherent conversation; the evening note describes moments of distraction. These are different observations at different times, not one uniform account of how you were throughout the period. [Source pages 10-11, 17-18]

Night entries generally say you stayed in your room or apartment and staff had no contact with you. That is not the same as staff confirming that you slept continuously.

The 6 August psychologist's note says you wanted to read the record in advance so that you could prepare and keep the discussion constructive. After the disagreement you brought her apple pie because you felt you had been too sharp. She explicitly describes this as an effort on your part to preserve a good treatment relationship. [Source page 19]

How the assessment changed over time

- **27 July:** After speaking with you and your mother, the psychologist's conclusion was that the changes appeared consistent with you becoming more like your old self. The plan was to keep talking openly. [Source pages 21-22]
- **31 July:** She found your speech more abstract and harder to follow. She questioned whether some claims about AI might involve grandiosity, while acknowledging your considerable skills. She wrote that early psychotic destabilization could neither be confirmed nor excluded at that point. [Source pages 20-21]
- **3 August:** She told you she thought you were becoming psychotic, while retaining recovery as an alternative explanation. The psychiatrist recorded "First signs of psychosis" and agreed to increase aripiprazole. [Source page 20]
- **6 August:** In the meeting with you and your mother, the psychologist said the signs were relatively subtle and could have other explanations. She said the team relied partly on intuition and professional judgment, and that they were not 100% certain. They chose to raise concerns early in the hope of preventing deterioration. [Source page 19]

- **7 August:** One nurse recorded no observed change and your report that you had noticed no effect from the medication increase. Another described good engagement and only brief distraction, but considered both recovery and behaviour resembling mania when discussing your future plans. [Source page 6]

Medication, monitoring and plans

- **Aripiprazole:** Increased from **2.5 mg to 5 mg daily on 3 August 2026**. The notes say you took the additional medication that day. [Source pages 10, 20]
- **Dexamfetamine:** The psychiatrist said to keep it unchanged for the time being. The dose is not given in this extract. [Source page 20]
- **Laboratory tests:** Annual testing was discussed. You initially hesitated or postponed, later agreed, and a 4 August nursing note records that you attended the lab. A 7 August note also says you had been that week. Results are not included. The mention of high cholesterol is a hypothetical example of a result you worried might lead to lifestyle pressure, not a recorded abnormal result. [Source pages 6, 9, 20-21]
- **Alcohol:** You reported recently having one or two beers by the water on three or four evenings per week. The psychologist connected the return of substance use with a past pattern of psychotic destabilization, while explicitly saying this did not mean it directly made you more psychotic. This is her documented discussion, not a conclusion that alcohol caused the current changes. [Source page 20]
- **Follow-up:** A medication review with psychiatrist Marijke Josephus Jitta was planned for **10 August at 10:00**, and an early-warning plan appointment with Jitske Koenders for **11 August at 11:00**. On 7 August the psychologist prepared existing plans for review and requested your recent records on USB. These pages do not contain the outcome of those appointments. [Source pages 2, 20]

Terms used in the translation

- **Psychotic symptoms / psychotic destabilization:** The record's language for suspected onset or worsening of psychosis. This extract references past psychoses but does not provide a full diagnostic history or establish a specific underlying diagnosis.
- **Grandiose ideas:** A staff interpretation that certain ideas about one's importance or achievements may be exaggerated. The source sometimes raises this as a question or impression, rather than stating it as established fact.
- **Maniform:** Resembling mania; translated here as "behaviour resembling mania." The word alone is not a confirmed diagnosis.
- **Signaleringsplan:** An early-warning and response plan: the document discussed in the record for recognizing changes and agreeing how to respond.
- **ADL / HDL:** Personal daily living activities / household daily living activities; often comments about personal care and the condition of the room.
- **Dagbesteding / DB:** Structured daytime activities, here involving ROADS Hoofddorp.
- **Bewindvoering:** Administration of finances by an appointed administrator. The extract does not set out its legal basis or conditions.
- **RIBW:** A supported housing service. The extract does not explain a specific offer or waiting-list position.
- **GZ-psycholoog / PSO:** Healthcare psychologist / psychologist. **PSA:** Psychiatrist. **VPK:** Nurse. **GGZ:** Mental health care/services. **MDO:** Multidisciplinary team discussion. **O.g. / OG:** The staff member writing the note. **Clt. / dhr.:** Client or patient / Mr.

Useful points to clarify with the team would be which observed changes they regard as specific warning signs, how those differ from your usual personality and thinking style, and what the medication review and revised early-warning plan concluded. Those answers are not supplied by this extract.

English translation of the record

The following pages preserve the record's order, which is grouped by report type and mostly runs from newer to older entries. Statements and first-person comments below are the original authors' reports translated into English.

Source page 1

Arkin Electronic Patient Record

Name: Klootwijk, T Patient number: 822577 Record number I70227739: 06-01-2022 to the present Date of copy: 10-08-2026

You are receiving a copy of the information held in your record at Stichting Arkin, where you are receiving, or have received, treatment. The record contains sensitive medical information; please handle it carefully.

Contents:

- 0001 - General progress notes
- 0002 - App progress notes
- 0003 - Clinical progress: goal-directed reporting
- 0004 - Clinical course/progress notes
- 0005 - Indirect group reporting

[Dates throughout this translation retain the source's day-month-year format. Repeated patient-identification headers, print footers, empty form fields, and empty question-mark placeholders are omitted.]

Source page 2

Patient: Klootwijk, T - 822577. Registration from: 06-01-2022.

General progress - Koenders, Jitske (healthcare psychologist), 07-08-2026 11:09:51

Entry date: 07/08/2026.

- Requested the records for the past 2 weeks on a USB stick through the service desk.
- Prepared an early warning signs/action plan for discussion next week, as previously discussed with the patient. All existing plans have been combined into 1 document for joint review.

Nursing: night shift - 01 Debby Luiting (nurse), 07-08-2026 07:09:01

Entry date: 07/08/2026.

The patient stayed in his room throughout the night; there was no contact with him.

"Kloof Varida" [as written; meaning or attribution unclear].

Nursing: night shift - 01 Debby Luiting (nurse), 03-08-2026 07:57:47

Entry date: 03/08/2026.

The patient stayed in his room throughout the night; there was no contact with him.

"kloof Varida" [as written; meaning or attribution unclear].

Nursing: night shift - 01 Debby Luiting (nurse), 02-08-2026 07:25:03

Entry date: 02/08/2026.

The patient stayed in his room throughout the night; there was no contact with him.

“kloof Varida” [as written; meaning or attribution unclear].

Nursing: night shift - 01 Debby Luiting (nurse), 01-08-2026 07:18:48

[Entry continues on source page 3.]

Source page 3

Continuation - 01 Debby Luiting (nurse), 01-08-2026 07:18:48

Entry date: 01/08/2026. Night shift.

The patient stayed in his room throughout the night; there was no contact with him.

“kloof Varida” [as written; meaning or attribution unclear].

Nursing: night shift - Aslami, Ahmad (patient support worker), 30-07-2026 05:13:13

Entry date: 30/07/2026.

The patient stayed in his room throughout the night; there was no contact with him.

Nursing: night shift - Aslami, Ahmad (patient support worker), 29-07-2026 05:57:28

Entry date: 29/07/2026.

The patient stayed in his room throughout the night; there was no contact with him.

Nursing: night shift - Aslami, Ahmad (patient support worker), 28-07-2026 05:14:41

Entry date: 28/07/2026.

The patient stayed in his room throughout the night; there was no contact with him.

Nursing: night shift - Aslami, Ahmad (patient support worker), 27-07-2026 06:05:08

[Entry continues on source page 4.]

Source page 4

Continuation - Aslami, Ahmad (patient support worker), 27-07-2026 06:05:08

Entry date: 27/07/2026.

The patient stayed in his room throughout the night; there was no contact with him.

Source page 5

Sunday day shift, temporary staff - Raspe, Martijn (mental health nurse specialist), 02-08-2026 13:15:03

A quiet shift; he was immediately the first to come for medication.

He goes in and out of the building very frequently. Otherwise, little of note.

Report: Jostha. [Attribution appears within an entry registered under Martijn Raspe.]

The patient interacted well, asked good questions in conversation and also gave answers. He did not come across as confused or incoherent in conversation.

Source page 6

Clinical progress: goal-directed reporting

The following observation points and goals are repeated in each clinical reporting form on source pages 6-18. They are translated here once and apply to all the entries in that section:

1. Are there changes in the psychiatric presentation?
2. How does the patient interact? What is his mood like?
3. How does the patient spend his day? Has he attended the daytime activity programme (ROADS Hoofddorp)? On Wednesday? No public transport tickets need to be provided.
4. Are activities of daily living and domestic activities in order? [Source abbreviations: ADL/HDL.]

Ham, Ruud (nurse), 07-08-2026 20:30:56

Goal-directed report:

Present at coffee time. Interacted appropriately and was able to follow the conversation well. He said he was briefly distracted once, but quickly recovered his focus.

In conversation, he said he was thinking about his future: he wants to end the arrangement under which his finances are managed, wants to look for a job, and, if RIBW does not provide something soon, wants to rent somewhere. [RIBW is not expanded in the source; housing is implied by the context.] He knows that this is expensive and thinks he will earn enough to pay for it. His account can, in a sense, be understood in terms of recovery; on the other hand, it could be interpreted as somewhat mania-like behaviour. He responds more quickly to ideas in the conversation and speaks more highly of himself than I am used to hearing from him. [The source wording here is awkward; this is the apparent meaning.] When I asked whether he was going to do anything else, he promptly replied that this was a good idea. Less than 15 minutes later, he came to let staff know he was leaving; he was going into town with his friend Deniz. He would send a message if he was going to be later than 00:00 or sleep elsewhere. He received permission months ago to decide spontaneously to stay elsewhere overnight.

Overman, Jostha (nurse), 07-08-2026 16:06:45

Goal-directed report:

No change in his mental state.

The patient says he has not noticed any effect from the increase in medication.

Otherwise, he interacts well, shows interest in the reporting staff member's health, and also participates in conversation at coffee time.

He went to the laboratory this week.

General progress:

His mother had ordered something for him. It had been delivered, and a colleague had accepted the parcel, but this was not known. The patient was looking for the parcel and became stressed. Eventually the parcel was found.

Linger, Brandy (group support worker), 06-08-2026 20:42:37

[The repeated observation points begin here; the report continues on source page 7.]

Source page 7

Continuation - Linger, Brandy (group support worker), 06-08-2026 20:42:37

Goal-directed report:

The patient was present at coffee time.

He said that he had had a meeting today with “pso” and his mother. [PSO is not expanded in the source.] Afterwards, he went out with his mother and they ate some fries together.

He said that he wants to cancel his AI subscription and put his VR headset up for sale on Marktplaats. He says he is not currently using either. He wants to focus less on the digital world and occupy himself with other things.

The patient is friendly in his interactions and easy to follow in conversation.

He looks well groomed.

Benschop, Menno (group support worker), 06-08-2026 09:50:16

Goal-directed report:

The patient was at the medication room for his medication at around 08:35. He had slept well and said he would be at coffee time at 10:00.

Zumbrink, Nanda (nurse), 06-08-2026 05:34:54

General progress:

The patient stayed in his room throughout the night; there was no contact with him.

Helsloot, Rakinah (community nurse), 05-08-2026 20:50:12

[Goal-directed report continues on source page 8.]

Source page 8

Continuation - Helsloot, Rakinah (community nurse), 05-08-2026 20:50:12

Goal-directed report:

1. The patient said he was tired from the daytime activity programme and therefore did not come to coffee time.
2. His interactions were focused on practical matters, and his mood was somewhat subdued.
3. He attended the daytime activity programme and was tired afterwards.

Linger, Brandy (group support worker), 05-08-2026 13:37:37

Goal-directed report:

The patient was not seen during the shift; he is at the daytime activity programme. [Source abbreviation: DB.]

Zumbrink, Nanda (nurse), 04-08-2026 23:45:00

General progress:

The patient stayed in his room throughout the night; there was no contact with him.

Ham, Ruud (nurse), 04-08-2026 21:22:52**Goal-directed report:**

Interacted appropriately at coffee time. A brief moment was observed in which his gaze drifted away and he was less focused on the conversation. This lasted only a few moments. Otherwise, he says he is feeling well. He is looking forward to the daytime activity programme tomorrow.

General progress:

He collected his medication for tomorrow because of the daytime activity programme.

Source page 9**Linger, Brandy (group support worker), 04-08-2026 08:44:34****Goal-directed report:**

The patient came for his medication in the morning and received it according to the medication administration chart. He told the reporting staff member that his medication had been increased.

He was reminded about visiting the laboratory. He expected that it would be busy given the time (08:35), and would prefer to go to the laboratory on Thursday. He said he would note this in his diary.

The patient is in a friendly mood and asks the reporting staff member how she is doing.

He looks well groomed.

He has been to the laboratory. [This statement is also present in the same entry, following the earlier plan to attend on Thursday.]

Zumbrink, Nanda (nurse), 04-08-2026 02:28:01**General progress:**

The patient stayed in his room throughout the night; there was no contact with him.

Ham, Ruud (nurse), 03-08-2026 22:07:47**Goal-directed report:**

Present at coffee time. Interacts appropriately. Appears to have difficulty focusing on the topic of conversation. Looks around a lot. However, his attention is readily engaged.

Overman, Jostha (nurse), 03-08-2026 16:23:15

[The repeated observation points begin here; the report continues on source page 10.]

Source page 10**Continuation - Overman, Jostha (nurse), 03-08-2026 16:23:15****General progress:**

The patient has received his medication. He has a meeting with PSO and PSA; see the report about this. [Neither abbreviation is expanded in this entry.]

Today, he started the increase in his aripiprazole medication from 2.5 mg to 5 mg. He has already received it.

He is to go to the laboratory tomorrow and is aware of this.

Lange, Jennifer de (patient support worker), 02-08-2026 21:40:53**Goal-directed report:**

1. The patient occasionally seems somewhat absent at coffee time and frequently watches the clock. He sometimes has a doubtful/pensive expression. [The Dutch “bedenkelijke blik” is ambiguous.]
2. The patient is friendly in his interactions.
3. During the shift, he largely does his own thing: he is programming or frequently goes outside for a walk.
4. Both are in order. [Refers to activities of daily living and domestic activities.]

Sieben, Eline (nurse), 01-08-2026 22:01:32**Goal-directed report:**

1. At coffee time, the patient looked a few times towards what appeared to be the ceiling? He also told the same story about his podcast twice. The reporting staff member will watch for this tomorrow. On those occasions, the reporting staff member was able to bring him back to the current conversation. He also asked how the reporting staff member was doing and about the family weekend she is organising. In this part of the conversation, he was again easy to follow. The reporting staff member will watch for this tomorrow and, if necessary, discuss it with him tomorrow.
2. His interaction varies; at times his thoughts seem to be elsewhere. His mood is good.
3. He walks a lot and listens to podcasts. He is occupied with a fantasy-related game. [The source does not specify whether he is playing or developing it.]
4. Both are in order. [Refers to activities of daily living and domestic activities.]

Source page 11

1 August 2026, 14:51:49 - Jostha Overman, nurse

Goal-directed report: He interacts well. No change in his psychiatric condition was observed (no strange, incoherent conversations/stories). His mood is normal. He did not state a plan for the day. Personal care is in order.

General progress: He was seen for his morning medication and took it.

31 July 2026, 20:57:15 - Kiki Mouw, group support worker

1. He attended the coffee gathering and was reasonably easy to follow. His contributions were mostly philosophical. He was able to keep his attention on the group conversation, although he occasionally took a turn that was somewhat difficult to follow.
2. He was friendly in his interactions and in a good mood.
4. Personal care was in order.

31 July 2026, 14:21:29 - Jostha Overman, nurse

General progress: He was seen for his medication, took it, and later attended the coffee gathering. He was given an information letter about Arkin's general house rules.

30 July 2026, 20:46:29 - Brandy Linger, group support worker

The report for this entry continues on source page 12.

Source page 12

Continuation: 30 July 2026, 20:46:29 - Brandy Linger, group support worker

He attended the coffee gathering. During conversations, he was sometimes difficult to follow. It was also observed that, several times, he looked at a fixed point for a while and did not respond immediately to the conversation. After some time, he rejoined the conversation. Sometimes he himself had lost track of the group conversation and asked another resident what it was about.

At a seemingly random moment, he asked: "Do you own shares?" Another resident responded by asking: "Are you investing in anything?" He answered: "I'm investing in my future," and explained that he had thought of products that could make a lot of money.

He was in a good mood and otherwise friendly in his interactions. He looked well groomed. Household activities were not observed.

30 July 2026, 11:41:55 - Menno Benschop, group support worker

No changes were observed. He was friendly in his interactions, came to collect his medication and then went outside. Personal care was in order.

29 July 2026, 21:16:03 - Kiki Mouw, group support worker

1. He was reasonably easy to follow in conversation and said a few things about the impact of his inventions. He himself joked that he did not have a "Messiah complex." He came across as appropriate in his responses.
2. He was friendly in his interactions and in a good mood.
3. He had attended the daytime activity programme. He said he was working on an augmented-reality project with a panda because he wanted to spend a little less time on technology and more on nature.
4. Personal care was in order.

Source page 13

29 July 2026, 14:37:39 - Nanda Zumbrink, nurse

He was not seen this morning. He was at the daytime activity programme today.

28 July 2026, 21:44:43 - Jennifer de Lange, client support worker

1. He seemed to be doing somewhat better compared with the weekend and seemed to have a somewhat clearer look in his eyes. No incoherent stories were heard.
2. He was friendly in his interactions and appeared somewhat more relaxed.
3. He had mostly been outside and walked a lot. He joined the coffee gathering and was pleasant company.
4. His room was not seen. He himself looked well groomed.

General progress: He was given his medication for tomorrow to take with him because of the daytime activity programme. His face was tanned, but his arms were sunburned. He was advised to use the sunscreen in the office tomorrow. After-sun lotion was also sought for him, but none could be found.

28 July 2026, 13:42:25 - Mireille Leysner, group support worker

1. No psychotic features were observed.
2. Friendly and open in his interactions. He said he was feeling better and better, and I complimented him on this. He appreciated it.

3. He was enjoying the weather, taking long walks and said he enjoyed them. He planned to...

This entry continues on source page 14.

Source page 14

Continuation: 28 July 2026, 13:42:25 - Mireille Leysner, group support worker

3. ...buy a cap tomorrow because of the expected heat.

4. He came on time for his medication and looked well groomed. His room was not checked.

27 July 2026, 21:08:21 - Ruud Ham, nurse

He attended the coffee gathering, interacting appropriately and focusing on the conversation as he used to.

27 July 2026, 09:33:32 - Brandy Linger, group support worker

He came for his medication in the morning and received it according to the medication administration chart. He appeared relaxed. He talked about his weekend and asked me how my weekend had been. He said he would listen to a podcast and work on projects today.

General progress - addition by Patricia: He joined the coffee gathering. We had an extensive conversation about various subjects, including the news, video games, AI and books. He also took the initiative in the conversation by asking me questions. We spoke in somewhat more detail about making the video game he is working on. He also enthusiastically showed what it currently looks like. He said the technical side came fairly easily to him, but he still found the game's story difficult. He said the conversation had rekindled his motivation to get on with it. We also discussed balancing the news with relaxation. He said he was mindful of this and tried not to let negative news weigh too heavily on him. He thanked me for the conversation and my enthusiasm, saying that it was infectious.

26 July 2026, 20:48:52 - Ruud Ham, nurse

The report for this entry continues on source page 15.

Source page 15

Continuation: 26 July 2026, 20:48:52 - Ruud Ham, nurse

He attended the coffee gathering. His interactions were appropriate. He came across as somewhat absent-minded and seemed preoccupied with other thoughts.

26 July 2026, 14:03:18 - Eline Sieben, nurse

1. Nothing unusual was observed.
2. Interactions and mood were fine.
3. He was walking outside and listening to a podcast.
4. Personal care was in order; his room was not seen.

26 July 2026, 06:40:49 - Adil Berri, nurse

General progress: He stayed in his apartment during the night. There was no contact with him.

25 July 2026, 21:50:50 - Ruud Ham, nurse

Friendly in his interactions. "Informative toward others" [the Dutch wording, "informatief naar anderen," is unclear]. Sometimes he seemed distracted in conversation, briefly looking in another direction and then seeming to need a moment to regroup. His interactions were appropriate, and he was generally easy to follow in conversation.

At the end of the conversation, he said he was working on something that would benefit humanity if he published it. He did not seem to want to be more specific about what he meant. At times he appeared to have somewhat grandiose ideas, which he then retracted or played down in the same sentence.

Source page 16

25 July 2026, 11:18:13 - Eline Sieben, nurse

He had not slept as well because he had been out. After his medication, he went back to sleep for a while. He planned to go for a walk in the afternoon.

25 July 2026, 06:16:34 - Adil Berri, nurse

General progress: He returned at about 23:50 and went straight to his apartment. He stayed in his apartment during the night. There was no contact with him.

24 July 2026, 20:47:43 - Kiki Mouw, group support worker

He was not in the building at the 20:00 and 22:00 “cm” [abbreviation not expanded in the source]. He was at the pub with a friend.

24 July 2026, 11:37:48 - Jennifer de Lange, client support worker

The report for this entry continues on source page 17.

Source page 17

Continuation: 24 July 2026, 11:37:48 - Jennifer de Lange, client support worker

He came of his own accord for his morning dose. He was friendly in his interactions and talkative at the coffee gathering. During the day, he did his own thing, including some programming. He had arranged to meet a friend in town this evening; they were going to the pub.

24 July 2026, 07:13:01 - Adil Berri, nurse

General progress: He stayed in his apartment during the night. There was no contact with him.

23 July 2026, 21:08:32 - Kiki Mouw, group support worker

1. In my opinion, his accounts were more difficult to follow. What he said mostly came down to people not being trustworthy, and he mentioned several times that he was responsible for what was done with his inventions. I told him that I was having trouble following him, that he seemed to be jumping from one subject to another, and that I was a little concerned, as agreed with him that he would be approached directly when there were suspicions of this kind. This made him somewhat angry, and he argued against it, but I also found him difficult to follow in doing so. Eventually he said that he could also simply keep quiet, and that Jitske (psychologist) would then be at his door tomorrow, just like last week. After that, he hardly looked at me anymore. The only thing in the others' subsequent conversation that he responded to was once again something from which he concluded that people were bad.

Earlier this evening, I was cooking with another resident. He entered the living room and walked around the kitchen island. He said, “You're in the way,” without looking at anyone in particular. When the other resident looked up (who was not in the way), he said, “Oh no, I wasn't talking to you,” and laughed. I was not near his path either. It was unclear what happened there.

My gut feeling was that things were not going well. I emailed the psychologist and explained this.

2. He was talkative, but there was less give-and-take than usual. He mainly talked at people. His mood was negative, especially after I raised my concerns with him. Even before that, he had mostly been negative about people and what was happening in the world.

This entry continues on source page 18.

Source page 18

Continuation: 23 July 2026, 21:08:32 - Kiki Mouw, group support worker

3. He said that after the coffee gathering he would sit by the water and listen to music. Asked what kind of music, he said "golden oldies." When asked what era that meant to him, he said, "From the time when I wasn't yet aware" [the meaning of "bewust" here is not clarified in the source]. He again launched into a stream of speech in which he said that music was no longer any good at all, but then said that there had never been a time when it was good, and he was again difficult to follow.

23 July 2026, 09:48:11 - Tess Koopman, student nurse

1. Nothing unusual was observed. His responses in conversation were appropriate and friendly, and he appeared relaxed.
2. He said he was doing well. He had remembered that today was my last day and asked about it with interest.
3. He said he had an appointment with the dental hygienist in Beverwijk today.
4. Personal care was in order.

General progress: He took his 08:00 medication as prescribed. He was given his 12:00 and 15:00 medication to take with him.

23 July 2026, 06:30:11 - Adil Berri, nurse

General progress: He stayed in his apartment during the night. There was no contact with him.

Source page 19

Progress: Conversation with the client and his mother

Jitske Koenders (healthcare psychologist / GZ-psycholoog) - 06-08-2026, 16:25:57

His mother says she was very shocked by our message to the client that we think he may be in the early stages of becoming psychotic. She refers to our telephone conversation last week, in which we discussed a very different hypothesis together about what is happening with Tom. She was shocked and also sad because she is afraid of what this will do to Tom's confidence in himself, and thinks it may also affect Tom's trust in us.

The client asks for a further explanation of what our view is based on. I tried to explain that this is also difficult because the client is certainly not severely psychotic, and that makes it difficult to identify specific behavior on which we base this. The things I mention could also be open to another interpretation. However, the fact that there had never previously been concerns about Tom during his stay here, from various people, and the combination of things we are seeing plus our experience with psychotic symptoms, nevertheless lead us to suspect that this is what is happening now. That does not mean we know it with 100% certainty or can completely dismiss Tom's and his mother's explanation: that he is recovering and returning to his old self.

The dilemma was whether to wait until things possibly get further out of hand, when there might be more certainty, or to have these conversations now precisely to prevent things from getting worse, though possibly with less certainty. We chose the latter. I said that I consider this good professional care. But the fact that the signs we are picking up now are relatively subtle means that, in explaining our reasoning, we have to rely more on our gut feeling and professional judgment. And that does mean another possible explanation can also remain alongside it.

Tom also explains that he is 100% convinced he is not psychotic, and therefore finds it difficult to be open to this explanation. We discussed that this was different in the past, and that I think if we had expressed our concerns some time ago, Tom would have responded very differently and perhaps been more open to our perspective.

Tom also says he would prefer me not to join the planned conversation with the psychiatrist [psa] on Monday. Tom further explains that it is not so much the medication increase that he has difficulty with, but mainly the fact that we are viewing him in this way. Evaluating his behavior during the medication reviews gives him a feeling of being in a 'trial period' and places him under closer scrutiny. This feels like a step backward compared with how things were before.

He also says he wants to request his record for the past 2 weeks, so that he can read exactly what has been recorded about him regarding the concerns we all have. He hopes this will give him some insight. When I suggest we could also look at the record together on the computer, he says he would prefer to read it himself beforehand and prepare that way, also so that he does not come across too sharply in the moment. He knows this can be a pitfall when emotions run too high, and he wants to keep the conversation constructive. We discussed that it is good that he recognizes this about himself, and that I will find out how best to arrange this for him.

I said that I also hope we are wrong and that it turns out nothing is wrong. The client and I both expressed the hope of eventually finding out the truth. We made a deal that if we turn out to be wrong, he gets apple pie from us, and vice versa.

Some time after the conversation started, Tom asks permission to record it, and says he now wants to do this more often so that he can defend himself if needed. We discussed that I and his mother consent to this, but I asked whether he would ask again each time. He agrees to do so.

At times during the conversation, the client can respond very angrily and sharply, and also says that he is angry. At those times he talks over my explanation. When this was pointed out to him and he then let me finish speaking, the conversation went better and became friendlier. Although we continue to disagree about the hypothesis that could explain the changes we see in Tom, we do manage to hear and understand each other's positions and end the conversation well.

Shortly after the conversation, the client comes by with a slice of apple pie, mainly to indicate that he felt uncomfortable about the conversation and thought he had been somewhat too sharp. I told him that I really appreciate the gesture. This also shows that, on his side, the client is making an effort to maintain a good therapeutic relationship, which is important for further progress.

Progress: Conversation with the psychologist [psa]

Jitske Koenders (healthcare psychologist / GZ-psycholoog) - 03-08-2026, 12:10:17

[The entry continues on source page 20.]

Source page 20

Continuation: Conversation with the psychologist - 03-08-2026, 12:10:17

- The client did not go to the laboratory this morning; he would prefer not to.
- We discussed that I was going to tell him something he would not like to hear: I think he is becoming psychotic. There is never a 100% guarantee. As far as we are concerned, the other hypothesis, that this is part of his recovery, still remains possible. However, given how I now know the client, if he asked me what I thought was happening, I would say that I think he is psychotic. The client's initial response is: fine, what are we going to do now? An early warning signs plan, more aripiprazole? Shall we go straight to the psychiatrist? Agreed.
- We then went to Marijke together; see Marijke's note. The client alternates several times between his initial attitude, 'I trust you, so what are we going to do about it now?', and a more skeptical approach:

'What are you actually basing this on? You are making statements like this on the basis of 2 conversations?' It is important to him that we allow both scenarios to remain possible alongside each other. That is fine. There is also no 100% certainty in these situations, and we emphasized that, in our view, we are still addressing this in good time.

- The client asks whether there are other things he himself can do to avoid becoming psychotic. We discussed maintaining a good sleep-wake routine, not using substances, continuing to eat and drink properly, and especially continuing activities. The client also mentioned that, recently, he has been drinking 1 or 2 beers by the water in the evenings, 3 or 4 days a week, for relaxation according to him. This is also relatively new behavior. In the past, psychotic destabilization also went together with substance use. We discussed this, but the client does not consider changing this a priority at present. It is also not the case that this directly makes someone more psychotic, but it does fit the previous pattern in which the client gradually starts using more substances again. This parallel was pointed out to him.
- Aripiprazole increased to 5 mg per day. I immediately went with the client to get this increased dose from the nurse. The client takes it there and then.
- Afterward, I sat down with the client briefly to check in. The client finds this an unpleasant message, and it brings back concerns: 'Will I be able to move away from mental health care [GGZ] in the future? How will this go in the future?' Understandable. I asked whether, when his mother comes here on Thursday, he would like the 3 of us to discuss this briefly. He does not feel a need for this himself, but he will tell his mother that the medication has been increased. If she does want such a conversation, he will let us know.

Medication review on Monday 10/8 at 10:00, with psychiatrist MJJ [Marijke Josephus Jitta] joining.

Early warning signs plan appointment with JK [Jitske Koenders] on Tuesday 11/8 at 11:00.

Progress: Medication

Marijke Josephus Jitta (psychiatrist) - 03-08-2026, 11:24:37

First signs of psychosis. Agreed to increase aripiprazole from 2.5 to 5 mg per day. Keep the dexamfetamine unchanged for now. Discuss again next week.

Progress: Conversation with the psychologist [pso]

Jitske Koenders (healthcare psychologist / GZ-psycholoog) - 31-07-2026, 12:09:35

The client says things are going well. He has not prepared a pitch; September is still too far away. Look at this further in due course.

The client is much more abstract in his thinking and speech than I am used to from him. He sometimes makes connections that are difficult to follow, and I really have to make an effort to understand him. I also discussed this with the client, who says that he can speak less abstractly but thinks he can speak this way with me. He also continues to maintain that this has been the case throughout his life and that he has always been something of an odd one out. We discussed that I notice I have to get used to it and really have to make an effort to follow him. We discussed that this must then have been a problem at other times in his life, which the client also acknowledges.

We discussed that a change in communication style, particularly thinking much more broadly and consequently using vaguer language, can be a sign of psychosis and of things going less well for many clients. According to his mother and the client, apparently that is not the case for him. We discussed that we...

[This sentence and entry continue on source page 21.]

Source page 21

Continuation: Conversation with the psychologist - 31-07-2026, 12:09:35

...will then need to draw up a new early warning signs plan together if this is indeed the client's new normal. The major challenge will be to distinguish between the broad, philosophical thoughts he already has now and the development of delusions.

The client talks about what he is working on: developing a new kind of AI, reflecting on the influence of conventional AI on humanity, and the technical side of the environmental problem. Particularly when he talks about these things, he remains vague and general. He says he is developing a product for which he has ordered materials, but cannot or does not want to say exactly what it is.

Later, the client says he would prefer not to have the annual laboratory testing scheduled for Monday 3-8, but he also does not want to go so far as to refuse it. He does not want to end up in a situation in which he might have to defend himself against possible findings from a test, for example that his cholesterol is too high and he therefore has to change his lifestyle. However, the client is already working on this, so what is the problem? That remains somewhat unclear. In the end, the client says it was good to have talked about it and that he will simply do it on Monday.

Observations: The client is harder to follow than on Monday. The client also says that he has invented a new kind of AI and wants to beat the computer. The client is of course very skilled, but the question does arise for me of whether this might also border on grandiosity.

Assessment [C/]: Is this a man whose baseline is a highly gifted / different / abstract thinking style, which is now returning after years of psychosis and medication? This is his mother's and the client's position. However, the transition then feels quite large. On the other hand, that also fits the client: he seems more often to suddenly 'flip a switch', including with other changes. Or are we actually looking at the beginnings of psychotic destabilization? I think we can neither rule this out nor confirm it at this time.

Plan [B/]: Stay in contact with the client and continue monitoring. If there are changes or increasing concerns, contact the client and his mother again about this.

Progress: Conversation with the psychologist [psol]

Jitske Koenders (healthcare psychologist / GZ-psycholoog) - 27-07-2026, 14:38:33

I discussed with the client that colleagues have been seeing many changes in him recently, and that this makes me wonder what colleagues are seeing and whether we should be concerned about it. I gave a brief summary of a few points I have heard repeatedly, but also said that this is not the whole story: it is also more about a feeling; interactions with the client are experienced differently. Whereas I never previously received comments or concerns about the client, I have now been receiving them for several weeks. It is good to explore this together.

The client also says that this is true: there have been many changes in his life recently, so we agree on that. He says that he himself sees this more as:

1. A new phase of recovery, focused more on self-reflection, but also on the future. The client is beginning to think about whether he might want children. But what kind of world would they be born into? He does not want to have a child out of selfishness, and wants to be able to stand behind that decision. But what, then, are his values and standards? The client says he is thinking about this a lot, something he has not done for a long time.
2. His emotions are starting to come back more after a long period of being absent. He himself notices that, because of this, the old Tom is returning, with both the positive and the negative aspects. Before he began experiencing psychoses, his emotional regulation was not always very stable either; for example, he could often feel panicky and anxious. He notices that emotions, both good and bad, are returning and that he has to learn to deal more with these old sides of himself that he had lost for so long.

We discussed that this can sometimes be difficult for us to interpret: we do not know that old Tom, only what Tom describes as the 'flattened version' of himself. Tom's mother knows both of these sides. Tom suggests calling her immediately: 'Then you know for sure that I have not coached her.'

We called his mother together on speakerphone and explained why we were calling and what our question was.

His mother says that she too clearly sees this change, but mainly views it as Tom's old traits returning. For example, she mentions sometimes not being able to follow him completely, something our colleagues also noticed, and says she recognizes this from the past: Tom is then already 3 steps ahead of her in his thinking. He talks much faster and more animatedly, and is thinking faster again too. This was always how he was, until he became ill. She also says that Tom can be very 'direct and sharp' in his communication, sometimes appearing somewhat irritated if something does not suit him. This was indeed also something colleagues noticed. According to his mother, this was also always the case in the past.

We do not know Tom this way at all, but apparently this does fit who Tom is as a person. We discussed that these are good, helpful examples.

His mother says she is not worried at all. She understands where our questions come from. She...

[This sentence and entry continue on source page 22.]

Source page 22

Continuation: Conversation with the psychologist - 27-07-2026, 14:38:33

...also needs a little time to adjust to the new situation, but mainly hopes Tom will not feel further inhibited by this from expressing himself to us. It is a shame that he is increasingly becoming himself and sometimes seems to be 'punished' for it. This also causes him frustration. His mother says she has a great deal of confidence that Tom will speak up if things are not going well. She notices that he is open and honest with her, including when things are going less well for a while.

Tom says that he really will tell us, and certainly me and his mother, if things are going less well.

I also went over a specific example involving a colleague with Tom. Tom also notices that he sometimes does not want to talk about things, out of fear that they will be misinterpreted, and then responds curtly. This is a new development resulting from a recently developed fear. Colleagues naturally notice this too and then wonder what is happening, which makes sense. On reflection, Tom does understand that his somewhat guarded attitude can also have this effect on others and, in doing so, reinforce this dynamic himself.

The client also says that he now actually dares to show that he is distracted. Previously he would have nodded along more and pretended he was following. He now responds less in a socially expected or accommodating way in that respect. So it is not that he is more distracted than before, but that he hides it less.

One final factor is that, in the past, changes in the client have always gone from 0 to 100 in a short time. Whereas changes in other people are more gradual, that has never been the case for Tom, including in the past. We discussed that a large change can sometimes set off some alarm bells for us, which Tom also understands.

We also discussed that we cannot always rely on someone coming to us themselves when things are not going well, because people sometimes no longer recognize that themselves. Tom then spontaneously gives some examples of where he draws the line between things being okay and no longer going well, and when he would speak up.

Assessment [C/]: All in all, there does indeed appear to be a change, but as far as I can assess at present, it fits with Tom increasingly becoming the old, and also slightly more imperfect, version of himself.

We agreed to simply keep talking openly about it with each other and with his mother.

Source page 23

Multidisciplinary team meeting [MDO] / indirect group report: Morning of 27-7-2026

Jitske Koenders (healthcare psychologist / GZ-psycholoog) - 27-07-2026, 09:07:49

Client report: Colleagues find the client different from before. The client responds with irritation when this is discussed with him. The way he spends his day has changed: he has stopped IT projects and is uncertain about day activities. He is more easily distracted and more easily irritated. His reaction is different: previously he was concerned about becoming psychotic, but now, when colleagues raise something with him, he responds with resistance rather than being worried about it.

The difficulty is that there are no very concrete indications of psychosis, such as delusions or hallucinations. However, several colleagues do have a feeling that there is now clearly something different in their interactions and communication with the client. It is difficult to pin this down to very specific things. All the individual changes in themselves are not particularly unusual or concerning; it is more the changing overall picture.

Plan [B/]: JK [Jitske Koenders] will discuss this again with the client today.